AC-05 CHF / Acute Pulmonary Edema

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use for adults age 16 years or older with suspected cardiogenic pulmonary edema or acute decompensated heart failure.

Early ventilation and perfusion pathway

1. Position + monitor	Upright unless hypotensive. Obtain continuous cardiac monitoring, SpO2, frequent BP, focused reassessment, and a 12-lead without delaying respiratory support.
2. Oxygen	Titrate to SpO2 92-96%; use 88-92% for known chronic hypercapnic respiratory failure unless severe hypoxemia requires a higher target.
3. Early CPAP	For moderate/severe distress with spontaneous breathing, airway protection, mask tolerance, and SBP at least 100 mmHg. Adjustable devices: start 5 cm H2O and titrate to 10 cm H2O.
4. Nitroglycerin eligible?	SBP at least 110 mmHg and no shock, preload dependence/RV infarction, or prohibited PDE-5 inhibitor use: 0.4 mg SL every 5 minutes, maximum 3 doses.
5. Shock or ventilatory failure	Withhold nitrates; reduce/remove CPAP if perfusion worsens; support ventilation. Avoid routine crystalloid in pulmonary edema.
6. Persistent cardiogenic shock	Paramedic: norepinephrine 0.1 mcg/kg/min IV/IO, titrate to MAP at least 65 mmHg or SBP at least 90 mmHg; maximum 2 mcg/kg/min.

Key treatment and handoff

Item	Approved action
Albuterol	Use the approved respiratory dose only when true bronchospasm is suspected; wheezing alone may be pulmonary edema.
Destination	Notify early, treat while moving toward transport, and target scene time less than 15 minutes.

Do not routinely administer furosemide, another diuretic, opioid, or IV-fluid bolus under AC-05.

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SAFETY, LINKED PATHWAYS, AND DOCUMENTATION

Safety checks

- Do not delay CPAP, ventilation, or transport for IV access or a 12-lead.

- Do not use CPAP for apnea, inadequate spontaneous ventilation, inability to protect the airway, active vomiting, or shock.

- Intubation in cardiogenic shock can precipitate profound hypotension or arrest; optimize oxygenation/hemodynamics and prepare vasopressor support.

Linked pathways

Pathway	Use when
AC-04	ACS/STEMI or acute coronary occlusion
AM-05	Persistent hypotension/shock
AR-03	Drug-assisted airway if CPAP fails
AC-03	Immediate pathway if pulseless

Documentation and release control

- Document the presenting condition, focused findings, rhythm/ECG when applicable, all interventions with dose/route/time/response, serial vital signs, reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.